

Schizophrenia: Patient Information Leaflet

A chronic psychotic disorder involving hallucinations, delusions, disorganized thinking, reduced emotional expression, and cognitive difficulties that affect daily functioning.

What is Schizophrenia?

Schizophrenia is a serious, chronic psychiatric disorder that typically emerges in late adolescence or early adulthood. It involves a combination of 'positive' symptoms — hallucinations, delusions, and disorganized speech or behavior that represent an excess or distortion of normal function — and 'negative' symptoms, such as reduced emotional expression, low motivation, and social withdrawal, which reflect a loss of normal function. Cognitive difficulties with memory, attention, and planning are also common and often have the greatest impact on long-term functioning. With modern treatment, many people achieve meaningful stability and quality of life, though outcomes vary considerably.

Common symptoms

- Hallucinations, most commonly auditory (hearing voices)
- Delusions — fixed, false beliefs such as persecutory, referential, or grandiose ideas
- Disorganized speech (loose associations, tangential or incoherent thinking)
- Grossly disorganized or catatonic behavior
- Flat or blunted emotional expression
- Alogia (reduced speech output)
- Avolition (reduced motivation to initiate or sustain purposeful activity)
- Social withdrawal and anhedonia

What causes it

Schizophrenia has one of the highest heritability estimates in psychiatry (around

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that only become clinically apparent after the brain's natural adolescent maturation and synaptic pruning processes. Environmental contributors include obstetric complications, prenatal infection or malnutrition, heavy adolescent cannabis use, urban upbringing, and migration-related social adversity, each of which appears to interact with underlying genetic vulnerability rather than cause the illness alone.

Treatment options

Antipsychotic medication is the cornerstone of treatment for both acute episodes and long-term relapse prevention, combined with psychosocial interventions such as CBT for psychosis, family psychoeducation, and social skills training. Specialized early-intervention services for first-episode psychosis have been shown to significantly improve long-term outcomes, making prompt assessment and treatment initiation especially valuable.

Self-help tips

- Maintain consistent medication adherence, even during periods of stability
- Avoid cannabis and stimulant use, which can worsen psychotic symptoms
- Keep a structured daily routine
- Stay engaged with family and support networks
- Attend regular follow-up appointments, including physical health monitoring given the metabolic risks (weight gain, blood sugar, cholesterol) associated with many antipsychotics

Prognosis

Outcomes in schizophrenia are variable: roughly a third of people achieve good functional recovery, a third have moderate improvement with some residual symptoms, and a third experience persistent, significant impairment. Early intervention meaningfully improves outcomes, while medication non-adherence is strongly associated with relapse. Life expectancy is reduced compared to the general population, largely due to cardiovascular and metabolic health effects and elevated

suicide risk, underscoring the importance of integrated physical and mental health

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When to seek urgent help

Seek emergency psychiatric care for an acute psychotic crisis with risk to self or others, command hallucinations instructing self-harm or harm to others, catatonia, severe self-neglect, or any suicidal ideation, which carries an elevated risk especially early in the illness course. A first episode of psychosis warrants prompt specialized assessment, since early treatment substantially improves long-term outcomes.

This material is for patient and family education only and does not replace individualized medical advice. For a personal diagnosis or treatment plan, consult Dr. Kushal Kharel or a qualified mental health professional.

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